

Benchmark Capstone Project

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Abstract

Blending art, science, theory, experience, worldview, and gained knowledge is what I have observed the best counselors aim toward. The importance of taking the best of what works empirically and experientially is a key to effective counseling. This paper is written to better understand the parts of theory, research, and practice that align with my worldview and true self. Once this better understanding is formed from peer-reviewed journals and primary sources, the blending of my findings is delineated. With as thorough a literature search as time would allow, my findings reveal solid evidence that my counseling theory and practice is built on empirically sound methods. My findings also reveal serious shortcomings in the science of my counseling which will be addressed with applications for mitigating these shortcomings.

Keywords: attachment theory, interpersonal theory, Sperry's model of case conceptualization, DSM-5, treatment planning, treatment outcomes

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Counseling as a profession is both art and science. The art of counseling comes in building relationships with each client individually. Therapy has a great deal to do with building a therapeutic bond that helps clients heal. The science of counseling comes in applying the theoretical and practical knowledge gained through research and education within said relationship. Blending the two is the start of forming healing relationships that are built on empirical understanding. Within this blending is yet more blending. In my training, I have heard that one theory should cover all disorders. I do not subscribe to that thinking, however. It is my understanding that an ethical counselor understands and applies the best theory and science for each disorder and each case. And even as one endeavors to find the perfect treatment for each case, the blending of theories typically finds its way into practice. It is difficult to find a theory of intervention, etiology, change, and/or success that does not borrow from other theories as it is. Blending what we know and what we are coming to know is what I'm finding counseling psychology to be about. What follows is an attempt to synthesize my ongoing blending of theory, assessment, diagnosis, treatment planning, case conceptualization, outcomes, and aftercare. I will begin by discussing the theories I apply in counseling.

Theories I Apply in Counseling

Attachment Theory

In truth, I have never searched for a theory of relationships nor change. Rather, working through my own emotional dysregulation brought attachment theory to me. What Rowe et al. concluded in their 2020 study was already real to me. They found that priming to attachment created secure attachments where none existed before. Creating secure attachments proved life-changing for me and I see the power of priming to attachment every day in practice. What this

study found was that attachment styles are valid predictors of how individuals regulate affect. Using assessments like Brennan et al. developed in 1998 helps me predict how clients react in dysregulating events and how to best help them regulate in the future. The assessment is a two-factor assessment for attachment style. The two 18 item self-report measures, conflated into one assessment, are a reliable way to measure clients for either anxious or avoidant attachment styles. According to Rowe et al. (2020), attachment mechanisms follow closely with a neurobiological understanding of hypervigilance and hypo-vigilance. Those with anxious attachment hyper activate to watch closely for signs of betrayal and abandon. Those with avoidant attachment do the opposite and shutdown emotional regulation. Secure individuals show affect out of emotional regulation. Attachment security priming shows benefits to emotional regulation, improving affect, and positive interpersonal effects according to the same study. Applying attachment theory in my practice allows me to ground clients, offer better ways to interact in important relationships, and give them tools they can use for a lifetime. When people feel secure in relationships, they feel they can guide their emotions rather than being ruled by them. When people feel they are guided by and guiding their emotions, they feel more at peace, they feel whole. Attachment theory helps me find the broken bonds from clients' past and guide them toward wholeness. I also use Attachment theory because blends well with the second theory I use in counseling.

Interpersonal

In recent years, Dan Siegel (2012) has built on the seminal work of Klerman and Weissman to better identify through neurobiology how healing through relationship is possible. When we repair out of a place of mindfulness, we repair out of something similar to secure attachments. What Dr Siegel refers to as a place of mindfulness, I often refer to as a place of

peace. Much like Attachment theory, Interpersonal Psychotherapy (IPT) gives me a structure from which I can lead clients to relational healing and wholeness. Being in a state of regulation enables us to heal trauma, repair broken bonds, and process distress and grief. Ravitz's and Watson's 2014 study showed that IPT is effective tool using relationship to have a positive effect on mental illness. Especially when Bowlby's attachment theory lens is the construct. My theories have been blended for some time I see. It is not a new discovery I have made, but rather a new understanding of what I knew to be true of these two theories as I practiced them and what others knew long before me. IPT is an effective and organic theory for a multitude of disorders. In a meta-analysis of seven major theories used for treating depression, Cuijpers et al. (2008) found that IPT was the most effective treatment studied. As depression or emotional distress is one of the most common disorders clients present with, I feel confident that applying IPT is valid. In addition to this study, according to a 1991 study by Klerman and Weissman, IPT is a highly effective treatment for individuals and couples with depressive symptoms and holds promise for many other disorders as well. I believe IPT is a part of many other theories because it is based on the commonly perceived basic functions of counseling therapy, therapeutic alliance, empathy, validation, present focused experiential relationship, is pragmatic enough to be generalizable, and gives clients experiences of a successful relationship (Markowitz & Weissman, 2004). Besides these factors, IPT is a natural fit for me. Dr Siegel's *The Mindful Therapist* is on my short list for reading soon. I hope that his book offers more insight into my own tolerance for compassion and my clients' and future students' tolerance for emotion as well. I tell my clients that I hope our work together is defined as a healing relationship. It is also helpful for me as a therapist since intentionally bringing clients to a place of mindfulness brings me to a place of mindfulness along with them. In other words, the art of counseling has come to me through my

own healing and attachment and IPT theories have been refreshing insight along the way. The science of counseling has not come so naturally. I have been surprised by what I found when researching the next section. This is where I have more work to do.

Comprehensive Assessment

Sperry's case conceptualization process has educated me to how assessment can be blended as well as theory. I will elaborate on the case conceptualization process shortly, but I do want to recognize how biopsychosocial, multicultural, and spiritual assessments match with Sperry's first category of case conceptualization. Sperry's first "P" of conceptualization is predisposition (Sosin, 2022). The implications of the predisposition phase of conceptualization are that clients come to us with factors that make them susceptible to mental illness. Using effective assessment for each client could be seen as the canvas for which we begin to paint a picture of their diagnosis and treatment, only, it is not a blank canvas. Blending the elements of comprehensive assessment help me see the big picture as it begins to reveal itself before me. A good biopsychosocial assessment is an important first step.

Biopsychosocial

Alfred Adler's Individual Psychology is in part based on the idea of holistic mental health (Shannonhouse et al., 2020; Sylvestro et al., 2021). What many have built on since Adler tells us that a view of the whole person at once brings a greater understanding of the person. If we focus too much on one part or fixate on treatment first, we lose sight of the nuances peoples' experience and environment bring to the canvas as we paint the picture. Biopsychosocial models began in the medical field and although assessments like wellness and quality of life have been found most useful in the medical field, there is evidence that metal health providers can benefit from and add to the understanding of these biopsychosocial models of assessment according to

Sylvestro et al. (2021). In times past, I viewed the benefit of assessment mostly in the scholastic setting. However, the above study states that biopsychosocial models of assessment are not only pertinent for assessment and research but for clinical practice as well. There are many published assessments for a biopsychosocial model, however I hope to find out more about Meyers' and Sweeny's 5 factor wellness inventory. According to Shannonhouse et al. (2020), Meyers' and Sweeny's 5 factor wellness inventory is a well-known and psychometrically sound assessment measuring creative self, coping self, social self, essential self, and physical self. I am intrigued by the concept of the parts of self in this model. I often use parts of self in therapy in ways similar to internal family systems. Being able to point back to common language with clients could prove insightful. In the past my biopsychosocial assessment has been primarily based on a few open-ended questions that I was curious about. This assessment has never been consistent from client to client however and is impossible to trace through treatment and prognosis. Having a valid model of assessment that points to multidimensional factors for clients would be like having a new paint color for the canvas. The above study from Shannonhouse et al. confirms multidimensional models as valid constructs for initial and ongoing conceptualization. Blending these three elements is in keeping with Adler's understanding of wellness and adds to my desire for wholeness for each client. They aren't the only factors that should be measured, however.

Multicultural

As I research empirical measures for how culture affects a client's mental health, two aspects are clear. First, we are multicultural beings. There are a variety of cultures and cultural influences that shape our development and behavior. In addition, we act and react neurobiologically to the different cultures we experience at any given time. So, to assume I do not understand another person's multicultural lens is both accurate and manageable. The second

aspect in play is that I do not automatically understand nor appreciate another person's cultural lens. I do my best to leave assumptions at the door when assessing a new client. But I am constantly at a loss for how to, delicately and appropriately, measure for how culture has shaped these people. According to Ridley et al. (2021), cultural competencies in counseling need much improvement. They sought to operationalize cultural competencies and found that clinicians were at a loss for how to define and practice multicultural counseling. The three main models currently in use are lacking and ineffective. This tells me that I am not alone in failing to understand how to better paint the picture of my clients' world. What I have found to be helpful to date is to listen well during other assessments for cultural factors that have played a role in the presenting problem of the client. As I listen to the family history and other factors that cause a predisposition to the mental health struggle, I take the risk to ask questions about cultural nuances that I am unfamiliar with. Similar to a biopsychosocial assessment that lacks repeatability and structure, this method allows me to remain relational in my approach to assessment albeit unscientific. In short, I am in search of a better method of measuring clients' multicultural experiences. Ironically, I have found a very scientific way of measuring and blending one of the most relational aspects of humans.

Spiritual

Traditionally, this is an area where I would push back hard for any scientific exploration or measurement. A person's spirituality is meant to be relational by nature. Maintaining the art of interpersonal relatability when it comes to transparently sharing on one's view of God or wounds caused by religion or religious people is a point of pride for me. Paradoxically, I have found an assessment that appears to be helpful in addressing my concerns with this factor of assessment. The God10 assessment has recently come on my radar as a valid measure for an individual's

view of God (Exline et al., 2015). This assessment measures the degree to which a person's struggle with God is causing negative views of God. There are questions in this assessment design to ascertain if a person views God as distant or loving or cruel. From there a clinician can make rational connections to faith, behaviors that are outside of a person's moral boundaries, and deeper connections to value and worth of self. In the spirit of the last statement, I feel as if I am constantly assessing for this connection. For instance, an individual's father wounds are fundamentally connected to his view of God and my radar is sharply tuned to listen for this connection. Also, important to watch for is the degree to which shame was used to manipulate an individual's moral compass. When people feel that have been duped into beliefs or behaviors limiting their autonomy, they don't often see God as loving. Religious institutions that call for legalistic moralism cause people to judge themselves by an unreachable standard and apply human burdens where they are not necessary. When this happens, clients struggle to find freedom to be true to themselves. These are experiences I am watching closely for throughout the counseling relationship because as our view of God changes so does our view of self and others.

Sometimes people's view of God changes so drastically that they no longer wish to include spirituality in their processing. You might think that these clients would steer clear of a counselor located in a church setting, but that is not true. Whether they are seeking to validate their wounds or find themselves without other resources, I am never surprised to see people show up that do not want to talk about God, at least not at first. So, assessing for willingness and openness is an additional part of spiritual assessment. Those who do not wish to discuss God or religion are no less welcome and no less spiritual. We are not spiritual beings because we want to be or not. We are spiritual beings and long for spiritual nourishment because we are created as such. I like to allow people to discover this on their own as they experience God's love through

our time together, however. All of this is located in the beginning stages of the counseling relationship for a reason. As Sperry (2005) delineates, the diagnostic question of what is asked first through the previously mentioned assessments to form a baseline of case conceptualization. The total picture of case conceptualization and the rationale are what we turn to next.

Case Conceptualization

Returning to the idea of art blended with science is the model of a client-centered case conceptualization. According to Sperry's article from 2005, this framework fits well with Adlerian wellness and interpersonal and narrative theories. It is not stated specifically, but, from the outline of the model, there is no reason attachment theory would conflict with this framework. I am drawn to this model of case conceptualization because of the blending I see with the other factors of assessment and overall treatment. Sperry's 8 Ps of conceptualization are an outline for assessment, diagnosis, treatment, outcome measure, and aftercare.

The first P of presentation is the communication of the clients presenting problem (although I don't like that term and I will use presenting perception or PP to describe what symptoms and complaints a client determines they need mental healthcare for during the remainder of this paper) through the intake process, assessment procedures, and observation (Sosin, 2022). As part of understanding and defining the clients PP, presentation takes the clients environment and relationships into account as a first impression of the diagnostic question of what (Sosin, 2022; Sperry, 2005).

The second P of predisposition seeks to explain the client's PP through factors that increase the susceptibility of mental illness. Assessments throughout the diagnostic formulation include family history, attachment style, loss, biological, psychological, cultural, and spiritual assessments (Sosin, 2022; Sperry, 2005). The above-mentioned assessments for biopsychosocial,

multicultural, and spiritual elements of a client's PP could be blended here to increase my ability to paint a full picture. The DSM-5 diagnostic process that will be discussed in more detail below is also a part of this first phase of conceptualization. Further blending the model within itself, the phases help me understand the model as a whole. The first phase, as such, includes presentation, predisposition, precipitants, protective factors, and patterns. The first three, including precipitants, come under what Sperry refers to as diagnostic formulations, or the what questions.

The third P of precipitants seeks to explain the client's PP through other extenuating and experiential factors such as physical illness, medications, family relationship distress, and other environmental stressors. Again, many of the assessments mentioned above are the measures intended to answer the what question of precipitants. The blended understanding of assessment gives me the answer to why we use assessment. When valid measures are used in a scientific manner, we are already in process of providing treatment through professional assurance to our clients. If an extenuating precipitant exists and a client can adjust medication, move out of a friend's house, enter a collaborative community, or undergo surgery to remove a pesky gallbladder, then our ethical responsibility to provide the best care possible, even if we never have one session of therapy, is achieved. Proper diagnosis does not always lead to my idea of therapy, but it I am not a counselor to practice therapy on those who need their wisdom teeth removed. But diagnostic formulations are not the end in themselves. As a counselor I need to move from diagnostic formulations to clinical formulations (Sperry, 2005).

The fourth P of protective factors and strengths moves me into the clinical formulations of the why question. It could still fit within the first phase (as I view them) as assessment is still underway, but with a different goal in mind. For this reason, protective factors and strengths, patterns, and perpetuates are in a second phase of clinical formulations (Sperry, 2005). Protective

factors sound like negative behaviors at first, but Sperry identifies these as ways the counselor can join with the client in finding a path to their own healing. Though Sperry tells me to ask why questions here, my first response is to ask what questions. What are the protective factors? I can see the value in asking why you can change, or why you have what you need to succeed. And I see the clinical aspect in the fact that I am joining with clients to attain their own goals. Here again the idea of interpersonal psychotherapy, attachment theory, and even some Rogerian client-centered theory is at play. Nothing in counseling happens in a vacuum. The blending, the integration of all these theories and models comes through best as I see the big picture of this case conceptualization model.

The fifth and sixth Ps are like one another. Pattern and perpetuates are both about maladaptive ways the client stays in a cycle of unhealth (Sosin, 2022). The first step is to identify and articulate the pattern and the second step is to ask more why questions about what keeps them in the pattern. I use the idea of pattern and cycle often in my session. It helps me to see the forest amongst the trees. Many times, a client is brought to emotional reaction when we articulate the training she has had through these patterns. Understanding that she has had a lifetime of returning to the model she was taught and that creating new healthy patterns is possible is insightful. The answer to why a client stays in a pattern is often related to the negative side of protection for me. Protecting in unhealthy ways is what I see usually keeps him returning to dysregulated reactions. Because of this, I often see treatment planning starting with these perpetuates. Conceiving of interventions that intersect a client's dysregulation and new ways of mindfulness and attachment priming is my way of interrupting old patterns and creating new one's with them, but more about this below.

The seventh and eighth Ps are plan and prognosis. The treatment planning process continues with plan. Once the assessments and diagnoses are evaluated, the third phase of treatment formulations or how questions is begun. Plan and prognosis are where my theories are blended into this conceptualization model. These theories guide the therapy phase of the relationship. They also aid in formulating my theory of change and therefore aid in addressing the prognosis of the client. It seems strange to me to offer a prognosis before treatment has begun. However, as I think through this model, it makes sense that if a client's prognosis includes how they will respond to an intensive inpatient treatment facility, for example, then my plan and treatment and relationship with this client will change early in the process. Seeing the end alongside the beginning is helpful as this model leads clinicians along a distinctly intentional path of blending art and science. Sperry's model is attractive to me because it allows me to blend in and interact with several different theories, assessment measures, and remain relational in the process. Something I have trouble conceiving of when it comes to the very scientific DSM-5 diagnostic process.

DSM-5 Diagnosis

To date I have held the view of diagnosis as overly medical, rigid, unrelational, and counter to my goals in counseling. To be structured to the point of missing the evidence that counseling is effective because we are empathetic, validating, building a model of relationship, and healing through experience is futility at its finest. On the other hand, to be unstructured to the point of being unable to describe a client's need for treatment and how to treat them is ineffective and unethical. My research is indicating that the best avenue for an accurate DSM-5 diagnosis is through the assessment measures provided by the American Psychological Association (APA) designed in conjunction with the development of the new manual. Many studies like the one

conducted by Bravo et al. in 2018 point to validity and reliability of the assessments. Yet, this is not part of my case conceptualization nor treatment planning. However, the intention of the APA to provide a systematic way for clinicians to diagnose and properly treat clients seems to be successful. Dailey et al. (2014) provide reason for trusting and utilizing the online assessments when diagnosing clients according to DSN-5 criteria. Using the structure of these assessments to pinpoint as precise a diagnosis as possible allows me and other clinicians to provide proper and effective treatment. The art of remaining relatable during this process is possible even though it is a very structured practice. From here, treatment planning can continue.

Treatment Planning Process

Treatment planning has not historically been a major emphasis of my work. For various reasons I have fallen out of competency with the skills built in graduate school and internship. The simplest versions and examples I have found for a treatment plan include a treatment goal, treatment objective, and interventions. Some models add steps to this plan by fleshing out the intricacies of the main three steps. I find this simple model to be useful and effective when faced with the practical time constraints of counseling practice. Remaining relational and continuing to blend art and science in this step consists of leading the client to her goals rather than stating them for her. If she feels that our first sessions are simply a rehearsal of my soon-to-come doctoral thesis, I fear I will drive her away from me and the helpfulness of counseling. So, I try to seek understanding and remain curious about why she come to call me. Finding treatment objectives has proven difficult for me in the past. The DSM-5 does not give treatment objectives in connection with diagnoses which makes it difficult to find objectives that are time-sensitive, measurable, and realistic. Antony and Barlow (2020) give good advice for specific DSM-5 related disorders and offer other sources for deeper dives into diverse perspectives. My research

is proving that an extensive library is not necessary for proper treatment planning. However, I need to find sources that help me with the dosage of treatment specifically. Continuing to utilize and understand Sperry's case conceptualization model will also benefit this area of treatment and the next area of outcomes assessment.

Outcomes Assessment

As a part of my research for this and other papers I came across the Outcome Rating Scale (ORS) developed by Scott Miller. According to a study by Miller et al. (2003), the ORS accurately measures changes in clients after interventions have been applied. The ORS is a brief measure that allows incremental application and little disruption of the relational style I like to focus on. I would like to find ways to blend this assessment into periodic sessions as I learn more about how to use it. The DSM-5 online assessment measures described above are also reliable sources for post-intervention assessment (Bravo et al., 2018). Although, as mentioned above the APA does not recommend the DSM-5 as a treatment planning tool they do recommend using the online dimensional assessments to measure changes in client's disorders (APA, 2022). Using these assessments to measure for the desired treatment outcome and meeting of treatment goals is an important part of the overall case conceptualization and clinical work. It not only validates the interventions and objectives chosen in treatment planning, but points to a plan for termination.

Aftercare

Thomas and Sosin (2011) reveal a great deal about the termination and aftercare process in their book *Therapeutic expedition: Equipping the Christian counselor for the journey*. Among the admonitions are considerations such as what it means to both the counselor and client to end a relationship. I have found this to be the main reason I extend treatment. I am hesitant to

terminate a relationship that I have invested in greatly and I am equally as hesitant to send a negative message about our relationship to the client. I appreciate the freedom Thomas and Sosin give in finding the good and healthy in termination. Although termination is not always up to me nor on my terms, when it is, I can help the client plan for long-term success building on our work together. Thomas and Sosin give several techniques for managing continued success. Making sure we acknowledge the challenges to managing mental health in a new season without a counselor, being realistic about unprocessed distress, and planning for a slow-down rather than a quick good-bye are all helpful suggestions. Overall, sending a client out with proper resources, an open invitation to revisit new or triggered emotions, and a support system to help with relapse prevention and relational health are my usual mode of operation. All the above information, practical knowledge and praxis is valuable. But fleshing this data out into a real-life situation is also helpful.

Case Study

Understanding how my theories, practice, and worldview are blended in my work with clients is best described in a narrative example of an actual case. To preserve ethical boundaries and maintain the confidentiality of my clients, it should be noted that this study follows one client in spirit, but the details and names are not representative of any client.

Demographic Information

Charlie is a 32-year-old male, married, Caucasian employed by a local church as a minister. He and his wife are expecting their first child in a few short months as we begin our relationship. Charlie has been an employee of the church for his entire adult life. In fact, he has been a part of this church his entire life. During High School Charlie interned for the church in the ministry area he is now employed full time for. During college, Charlie was working toward

the goal of marriage to his wife of 3 years and a full-time position at the church. Since graduating Charlie has held several positions at the church. Charlie's family of origin consists of his married mother and father, his two brothers (one older and one younger), and his older sister. Charlie is healthy physically and lives a relatively content lifestyle.

Presenting Perspective

Charlie is relatively happy in his relationships and his work. The only distress in Charlie's life is his inability to give up looking at pornography and masturbating. This habit is causing depression, shame, and an inability to believe he is performing his job well. More about Charlie's presenting perspective can be found in Appendix A where I delineate his case conceptualization.

Observations

Charlie presented to me well dressed and groomed. This was not his first counseling intake, so he was admittedly prepared, and I observed that readiness. He made appropriate eye contact and was able to answer questions without stumbling or stuttering. His cognitive processing was observably intact by his follow-up questions to me. Charlie was a little out of shape and admitted that he had gained some weight as he worried about being caught looking at pornography. Charlie was oriented to his surroundings, and I was not concerned about psychosis nor tangential thinking.

Charlie's Story

Charlie contacted me as a part of the same church staff and a previous knowledge of my ministry. He texted me that he was having some old problems arise and wanted to meet and see how I could help. Our initial meeting consisted of a brief intake questionnaire review and an informed consent review. After agreeing to and signing the consent, I ask him to tell me his

reason for seeking counseling. My goal was to set him at ease by giving him the opportunity to tell his own story. Charlie told me that his struggle with pornography started when he was sexually abused by a neighbor boy a little older than him when he was seven. He never reported this abuse and he and his family never addressed it as a child. When he became an adult and chose to seek help from another counselor, the abuse surfaced and only then was it processed and connected to the unwanted sexual behavior. Charlie told me about his idealistic childhood, parents who loved and supported him throughout development, and siblings who had disagreements but learned to love each other. Charlie told me then that he had struggled to avoid using pornography as a means of coping since High School. Once exposed by some friends' playful banter about how to find it on the internet, he couldn't remember a time without images and videos depicting sex of some kind in his life.

Continuing Charlie's history and assessment transitioned into the second session as usual with my clients. Hearing how masturbation had played a role in Charlie's development and understanding of sex led me to believe that the newest considerations for a diagnosis of hypersexuality may be in order. I thought so because of the criteria of increasing impulsive use of sexual behavior. These behaviors should also be connected with fantasy, arousal, and urges. Additionally, hypersexuality is a consideration when these acts are not diminished by adverse consequences (Kafka, 2010). Charlie then told me that college and pre-marriage had been more of the same, seeking out sex in some form or fashion to escape the pressures of school and life. Anxiety was beginning to surface because of pornography use as well as depression.

Consulting with Charlie on what our goals for counseling should be we reached a consensus that eliminating pornography use and masturbation was the one goal he desired to accomplish. At this point I made three suggestions for our ongoing relationship. First, I asked

him to join a community of men struggling with using sex as a false connection. These men would offer accountability as well as safety in transparently sharing his story. Secondly, I asked him to consider reintroducing his wife to his struggles. I offered a plan to discover the why of his pornography use and safely invite his wife to hear this insight and ask questions about it. Thirdly, I discussed meeting weekly for eight weeks to begin processing his story. When I ask clients to get to the bottom of the why I am asking them to discover the reasons they have for escaping with the behavior we are trying to eliminate.

As Charlie agreed to all three of these suggestions, our counseling therapy began in earnest. The next week we began to practice mindfulness-based relaxation techniques which were designed to help Charlie access the part of his self that wanted to escape. In addition to this, we continued some attachment style assessment with the goal of seeing the broken bonds and eventually priming to attachment for connection.

As we progressed through the weeks Charlie began to discover that his pornography use had two main uses. First, he discovered that he was able to escape the anxiety caused by past shame of his sexual abuse and the embarrassment it caused him in romantic relationships. Second, he discovered that he had a deep felt need of being wanted by a woman. The fantastic realm of pornography allowed him to place himself in the position of a man receiving the desire of women.

With these two insights in mind and the attachment wounds of his father's perfectionism and his mother's coldness, we primed to attachment of love, acceptance, and desire. Helping Charlie see through the experiential relationship we were developing that he could be imperfect and still receive love was moving the needle towards openness with his wife. Priming to attachment where sexual desire is normal and natural also helped Charlie discover his arousal

was not disgusting. As well as these constructs and the interpersonal processing we used to navigate the shame and incongruent behavior, we primed to attachment with his wife as a safe person to share arousal and desire with.

Through all this Charlie's self-report of frequency of pornography use and feelings of anxiety and depression were used to measure change and the effectiveness of the treatment. Over the sessions depression and anxiety began to change first. Charlie reported having more days of joy and more overall enjoyment of life. He also reported that the mindfulness exercises were helping him communicate with his wife, employer, and friends. After meeting with Charlie for the eight sessions and seeing success, it was time to invite his wife into the process.

Charlie, his wife, and I spent three sessions disclosing his history in a safe and general manner, allowing her to ask clarifying questions, and building bonds that would help them both heal in the future.

Once Charlie and his wife were both at a place where the pornography use had stopped, Charlie was still utilizing his support system for accountability and transparent interpersonal sharing, and she was comfortable moving forward we started introducing termination. My first session with Charlie after deciding on a termination plan was spent discussing the ways he had improved and grown. We talked about the system he had in place and how they were there to help him continue to succeed. We then decided to arrange four follow up sessions spaced out every other week. During these sessions we continued to process desire and anxiety with an interpersonal dynamic and prime to attachment if new wounds came up. Experiencing these ways of healthy relating led us to fully terminate after these four sessions.

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Appendix A

Charlie's Case Conceptualization

Through the lens of Sperry's model (2005), Charlie's case makes sense. The 8 Ps of the model paint a clear picture for the what, why, and how that Charlie needs. The first P of presentation offers the presenting problem of depression and anxiety caused by problem pornography use. At first Charlie would have said the problem is only the pornography, but during assessment we discovered together that his distress was more of a problem than previously thought.

Charlie had both Predisposition and precipitants as he sought me out. His predisposition was seen in childhood sexual trauma and a lack of support from anyone to make sense of this abuse as he developed through life. In addition to this, his seemingly perfect family life caused him to wonder why he couldn't have the determination his father had to say no to unwanted and perceived sinful behavior. His mother's coldness in relating to him also created several attachment wounds and caused him to wonder about connection with women. The precipitants were a high stress work environment, and secrets he was keeping from his wife and friends and support system. Also seen as a precipitant was the shame caused by repeated pornography use incongruent with his moral system. The diagnosis that is emerging for this type of case is hypersexuality. Although I am reluctant to give any formal diagnosis, delineating the criteria of increasing maladaptive use of impulse sexual behavior especially when these behaviors are in response to fantasy, arousal and urges and not curbed by adverse consequences (Kafka, 2010) was helpful for Charlie.

Protective factors and strengths that Charlie held were his ability to empathize with others struggling with emotional dysregulation and incongruent behavior. His empathy allowed

him to see himself as he would see others. We could use this empathy to interpersonally regulate his system as he struggled to know how to change. His protective factor of love and care for his wife was also a primary reason he sought help. Not wanting to hurt her any more than he had would be a motivational factor.

The pattern Charlie had was almost obvious. In fact, he described his pattern of stressing out, feeling alone, and acting out on our first session. The sexual addiction cycle is a pattern that is recognizable and nearly universal. From sobriety to preoccupation, to ritual, to acting out, to shame and despair, to sobriety we can follow Charlie's maladaptive way of escaping reality. In addition, his perpetuates are seen as part of the cycle. More than this, shame and moral incongruence became the two main perpetuates (Grubbs et al., 2019). His use of pornography to numb the feeling of shame and disdain made him feel ashamed.

Our plan was to use attachment theory and interpersonal psychotherapy in individual sessions over eight weeks to discover, articulate, and dismantle this cycle. External resources of his relationship with his wife, a community of men struggling with the same issues, and new relationships that would help him navigate to healthy transparency and attachments were also employed. During these eight sessions we would prime to attachment in connection to intimacy and desire to build new bonds of security. We would also process his story interpersonally allowing relationship to heal attachment wounds. Mindfulness techniques directed at interrupting his neurological reactions would bring awareness to when he wanted to use pornography to escape, how the desire could be met in other healthier ways and ease the anxiety around old wounds.

Charlie's prognosis was hopeful if he stayed active in community and learned to connect in authentic intimacy with his wife and other safe attachment figures. It is not uncommon in

these cases to experience a relapse or multiple relapses. Sexual addiction treatment is not linear, but often circular like the cycle the client is in. Charlie continues to stay connected to his relational community, grow in connection and transparency and honesty with his wife, and check in with me periodically when old triggers ask him to act out in old ways.

Appendix B

Charlie's Treatment Plan

Problem or Concern	Measurable Treatment Goal	Treatment Interventions	Expected number of sessions devoted to reaching goal	Measurable means of evaluating progress towards goal	Aftercare
Unwanted Pornography use	Replace use with relational connection	Interpersonal relating in sessions where immediacy and use of the self of counselor are priming to attachment and creating healthy models of relationships. Group sessions with a community of men willing to use their own stories to create healing in shame messages. Couple's sessions with his wife to disclose previous behavior and prime to attachment with her as a safe figure.	I expect 8 individual sessions and 3 couple's sessions 4 follow-up sessions and ongoing group work.	Self-reporting each session on frequency of use	4 follow-up sessions designed to incrementally release Charlie to his external support system

Anxiety	Decrease anxiety	Mindfulness techniques designed to regulate Charlie and give him an understanding of how to bring himself to a place of peace. Use breathing, visualization of safe, peaceful people and places, use guided imagery and grounding techniques to experientially place Charlie in a regulated state.	3 sessions of mindfulness followed by outside work with podcasts and reading to increase Charlie's knowledge of mindfulness.	Charlie's self-report of anxious feelings and times when he thought he needed pornography to soothe decrease over each session. Use the Brennan assessment to measure for secure attachment styles (Brennan et al., 1998)	Continue to grow in mindfulness and connection with spouse.
Depression	Increase days of joy and overall satisfaction with life and relationships.	Prime to attachment through experiential interpersonal relating and immediacy in session. Noticing Charlie's view of self and priming to secure figures	8 sessions of priming to attachment.	Charlie's self-report of more days of joy and overall satisfaction with life	Occasional check-ins when Charlie is feeling discouraged to help him prime to attachment again.

		to help him understand he is okay in the hard times.			
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This is an engaging, well-written paper. It evidences that you met the learning outcomes designed for this course.

May you have much success on the QE Raymond! 😊